

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Dale R. Whitcombe DOB: 1955-06-21 (age 71) Member ID: MBR-40217

Sex: M

Requesting Provider

Practice: Larkspur Valley Heart & Rhythm

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-07

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Electrophysiology consultation

Larkspur Valley Heart & Rhythm - EP Clinic

Pt: WHITCOMBE, DALE R DOB 06/21/1955 MRN LV-552310

DOS: 09/22/2026 Referring: K. Abernathy-Ruiz MD (general cardiology)

Reason for consult

Primary prevention ICD eval, ischemic CM.

HPI

71 yo M w/ CAD, insulin-requiring DM2, CKD 3b, HTN. Admitted 08/27/2026 w/ NSTEMI (peak hs-TnT 1,840 ng/L). LHC 08/28 w/ severe 3V CAD incl chronic total occlusion of RCA filling via L-to-R collaterals and diffuse mid/distal LAD dz. Heart team reviewed films: poor distal targets, CABG not offered, no PCI attempted. Medically managed and d/c'd 08/31 on DAPT + GDMT.

Since d/c gets winded walking up his driveway or carrying groceries in from the car. Comfortable at rest, sleeps on 1 pillow. No syncope, no presyncope, no palpitations.

Data

TTE 08/28/2026 (inpatient): LVEF 25%, inferior/inferolateral akinesis.

Repeat TTE ordered >30d post event.

ECG today: SR 74, QRS 118 ms (nonspecific IVCD), inferior Q waves.

Assessment

1. Ischemic cardiomyopathy / HFrEF. NYHA class II.
2. CAD, NSTEMI 08/27/26, medically managed, not revascularized.
3. DM2 on insulin, CKD 3b (Cr 1.8), HTN.

Plan

- Single chamber ICD if repeat LVEF stays $\leq 35\%$ after the post-MI waiting period. QRS < 130 , not a CRT candidate.
- Refer to HF NP for formal SDM visit w/ decision aid before we book.
- Do NOT interrupt ASA/ticagrelor peri-procedure.

Electronically signed: Priyanka Oduya-Lind, MD, FHRS 09/22/2026 16:42

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Hospital discharge summary

MERCY LARKSPUR GENERAL HOSPITAL - DISCHARGE SUMMARY

Patient: Whitcombe, Dale Admit: 08/27/2026 Discharge: 08/31/2026

Attending: S. Mulrooney, MD (Cardiology)

Principal dx: NSTEMI (I21.4)

Secondary: CAD native vessels, HFrEF new dx, DM2 w/ hyperglycemia, CKD 3b, HTN

Hospital course: Presented w/ 3h substernal pressure. Trop rising, peak 1,840. Started heparin gtt, ASA, ticagrelor load. LHC 08/28: LM nl; LAD diffuse 70-80% mid and distal, small caliber; LCx 50% OM1; RCA CTO proximal w/ L-to-R collaterals. No intervention performed. CT surgery (Dr. Hanlon) reviewed: poor targets, high STS risk, not a surgical candidate at this time. TTE 08/28: LVEF 25%, inferior/inferolateral AK, mild MR. Pt has no prior PCI or CABG.

GDMT started prior to d/c, tolerated w/o hypotension. Cr stable 1.7-1.9.

Discharge meds: see med rec.

F/u: Cardiology 2 wks, EP referral for ICD eval after waiting period.

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Transthoracic echocardiogram

LARKSPUR VALLEY HEART & RHYTHM - ECHO LAB

Study date: 10/01/2026 Pt: Whitcombe, Dale R Ht 70 in Wt 204 lb BP 118/66 HR 71

Indication: ischemic CM, reassess LV function post-MI

Comparison: 08/28/2026 inpatient TTE

Measurements:

LVIDd 6.1 cm LVIDs 5.0 cm IVSd 1.0 cm

LVEF (biplane Simpson): 30%

TAPSE 1.9 cm RVSP 34 mmHg LA vol index 41 mL/m²

Findings: Moderately dilated LV. Akinesis of the basal-mid inferior and inferolateral walls, hypokinesis elsewhere. Moderate global systolic dysfunction. Grade II diastolic dysfunction. Mild functional MR. Normal RV size/fxn. No pericardial effusion. No LV thrombus.

Impression: LVEF 30%, modestly improved from 25% on 08/28/26. Regional WMA as above.

Read by: T. Ingersoll, MD 10/01/2026

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Medication reconciliation

Medication List - reconciled 09/22/2026 (EP clinic)

Whitcombe, Dale R DOB 06/21/1955

Medication | Dose | Start date | Indication

Aspirin | 81 mg daily | 04/2019 | CAD

Ticagrelor | 90 mg BID | 08/27/2026 | ACS

Metoprolol succinate | 50 mg daily | 08/30/2026 | HFrEF

Sacubitril/valsartan | 24/26 mg BID | 08/30/2026 | HFrEF

Eplerenone | 25 mg daily | 08/30/2026 | HFrEF

Empagliflozin | 10 mg daily | 08/30/2026 | HFrEF / DM2

Atorvastatin | 80 mg daily | 04/11/2019 (dose incr 08/2026) | CAD

Insulin glargine | 28 units qHS | 2016 | DM2

Insulin lispro | sliding scale AC | 2016 | DM2

Furosemide | 20 mg daily PRN wt gain >3 lb | 08/31/2026 | volume

Allergies: lisinopril (cough)

Reconciled by: J. Tolliver, RN

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Shared decision making visit

HEART FAILURE CLINIC - SHARED DECISION-MAKING ENCOUNTER

Date: 09/29/2026 Pt: Dale R. Whitcombe Accompanied by: wife (Joan)

Clinician: Renata Aguillard, ACNP-BC

Purpose: SDM re: primary prevention ICD prior to implant, as required for device candidates.

Tool used: ICD patient decision aid (evidence-based, booklet + 12-min video), reviewed together in clinic.

Covered: estimated SCD risk with LVEF $\leq 35\%$; what an ICD does and does not do (does not treat HF sx); inappropriate shocks; lead and infection risk; driving restrictions after shocks; option of deactivation at end of life; alternative of continuing medical tx alone.

Pt teach-back accurate. Values: wants to see granddaughter graduate, accepts risk of shocks. Wife supportive.

Decision: pt elects to proceed with ICD. Copy of decision aid given.

Time: 40 min, >50% counseling.

R. Aguillard, ACNP-BC 09/29/2026

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Pre-procedure labs

LARKSPUR OUTPATIENT LAB Collected 09/30/2026 07:52

Pt: WHITCOMBE, DALE Ordering: P. Oduya-Lind MD

Na 138 K 4.6 Cl 103 CO2 24 BUN 31 (H) Cr 1.8 (H) eGFR 38 (L) Glu 188 (H)

Mg 2.0

WBC 7.2 Hgb 12.1 (L) Plt 214

PT 11.9 INR 1.0

NT-proBNP 1,612 pg/mL (H)

HbA1c 8.4% (H)

TSH 2.1

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Pre-admission procedure plan

EP PRE-ADMISSION H&P / PROCEDURE PLAN

Date: 10/02/2026 Pt: Whitcombe, Dale R Proc: transvenous single-chamber ICD (CPT 33249)

Planned admission: 10/07/2026, inpatient

Indication: ischemic CM, LVEF 30% (TTE 10/01/26), NYHA II, on GDMT, SDM completed 09/29.

Interval hx: no chest pain or ACS since index NSTEMI of 08/27/26. No revascularization done or planned.

Expected length of stay: 2 midnights.

Rationale: (1) uninterrupted DAPT w/ ticagrelor + ASA <6 wks from ACS, high pocket hematoma risk, needs serial pocket checks and compression; (2) CKD 3b, eGFR 38, renal fxn and K check POD1; (3) insulin-requiring DM2 w/ A1c 8.4, needs inpatient insulin titration around NPO period.

Screening for contraindications: alert, oriented, manages own meds, no cognitive impairment. No malignancy or other illness limiting survival to <1 yr. No AF history, SR on all ECGs. Euvolemic, BP stable on oral regimen, clinically stable for elective implant.

P. Oduya-Lind, MD 10/02/2026

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